

BEYOND BREAST CARCINOMA: BREAST MALT LYMPHOMA DETECTED ON SCREENING MAMMOGRAPHY—MULTIMODALITY IMAGING AND STAGING CORRELATION

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INTRODUCTION

Breast lymphoma is a rare extranodal malignancy, accounting for less than 1% of breast malignancies. Extranodal marginal zone lymphoma of mucosa-associated lymphoid tissue (MALT lymphoma) is an uncommon subtype with non-specific imaging features that frequently mimic primary breast cancer. We present a screen-detected case highlighting the complementary roles of multimodality imaging, histopathological diagnosis and multidisciplinary management.

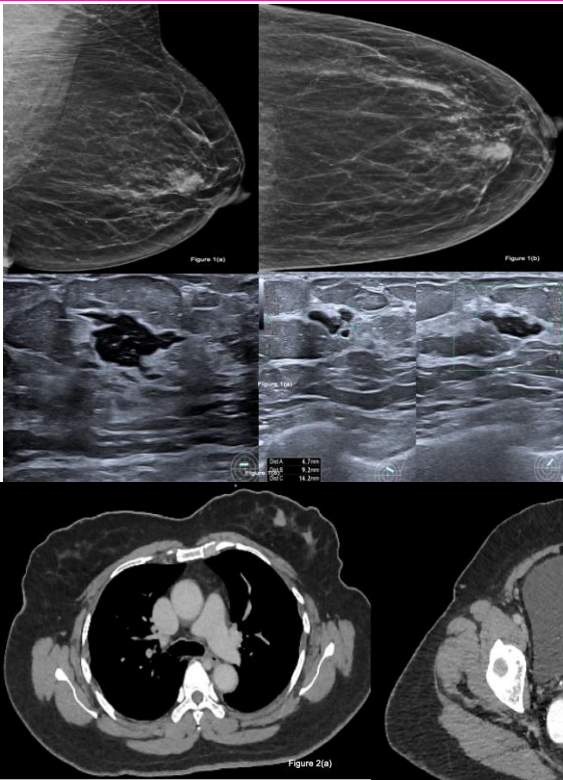
CASE PRESENTATION

A 55-year-old, asymptomatic postmenopausal woman with low risk factors for breast carcinoma, underwent baseline screening mammography before commencing hormone replacement therapy.

HISTOPATHOLOGY

Histopathology and immunohistochemistry confirmed extranodal marginal zone lymphoma of mucosa-associated lymphoid tissue (MALT lymphoma). Following multidisciplinary tumour board discussion, the patient was referred to haematology for definitive management.

IMAGING FINDINGS



Mammogram. Fig 1(a) and (b) depicting lobulated equal-density lesion in the left upper inner breast.

Ultrasound Fig 1(c) and (d) revealed two lobulated heterogeneous hypoechoic masses measuring 3.1 cm and 1.4 cm each.

Fig 2(a) depicting the two breast lesions without chest wall invasion and Fig 2(b) demonstrating a solitary enlarged left external iliac lymph node suspicious for nodal involvement

DISCUSSION AND CONCLUSION

Breast MALT lymphoma is an uncommon but important differential diagnosis for suspicious breast lesions. Although imaging findings overlap with breast cancer, histopathological confirmation is essential. Cross-sectional imaging plays a pivotal role in staging, detecting occult extra-mammary disease and guiding lymphoma-directed management while avoiding unnecessary breast surgery.

Breast MALT lymphoma should be considered in the differential diagnosis of suspicious screen-detected breast lesions. Multimodality imaging, histopathological confirmation and multidisciplinary collaboration are essential for accurate diagnosis, comprehensive staging and appropriate patient management.

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